

CASE 1 – FAMILY DOCTOR FOLLOW-UP

Chronic Asthma + Allergic Rhinitis + Recent Worsening of Symptoms

(Long-form Hinglish consultation designed for medical transcription testing)

Patient Details

Patient Name: Shabana Khan

Age: 42 years

Gender: Female

Visit Type: Follow-up Visit (Known Patient)

Known History:

- Bronchial Asthma for 12 years
 - Allergic Rhinitis
 - Seasonal exacerbations during monsoon and winter
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Doctor–Patient Conversation

Doctor: Assalamualaikum Shabana ji, kaise ho? Pichli baar toh breathing kaafi better ho gayi thi.

Patient: Walekum salam doctor saab. Haan better thi, lekin pichhle 15-20 din se phir problem shuru ho gayi.

Doctor: Kaisi problem ho rahi hai?

Patient: Saans phool rahi hai. Especially raat ko aur subah uthte waqt.

Doctor: Khansi bhi hai?

Patient: Haan. Dry cough zyada hai.

Doctor: Balgam aata hai?

Patient: Kabhi kabhi halka sa safed balgam aata hai.

Doctor: Fever hua tha?

Patient: Nahi.

Doctor: Cold ya viral infection hua tha recently?

Patient: Mere bete ko hua tha. Mujhe bhi halka sa sardi-jukham hua tha pichhle hafte.

Doctor: Theek. Rescue inhaler kitni baar use karna pad raha hai?

Patient: Roz lagbhag 2 ya 3 baar.

Doctor: Roz?

Patient: Haan doctor.

Doctor: Ye toh kaafi zyada hai.

Asthma Control Assessment

Doctor: Raat mein kitni baar neend khulti hai breathing ki wajah se?

Patient: Hafte mein 3 ya 4 baar.

Doctor: Walk karte waqt problem hoti hai?

Patient: Pehle nahi hoti thi, ab thoda fast chalne pe saans phoolti hai.

Doctor: Stairs chadhne pe?

Patient: Do floor chadhne ke baad rukna padta hai.

Doctor: Wheezing ki awaaz aati hai?

Patient: Haan, especially raat ko.

Medication Compliance

Doctor: Controller inhaler regular le rahi ho?

Patient: Doctor saab honestly bolu toh kabhi kabhi bhool jaati hoon.

Doctor: Kabhi kabhi matlab?

Patient: Week mein 3-4 din miss ho jata hai.

Doctor: Aur rescue inhaler regular use kar leti ho.

Patient: Haan kyunki usse turant relief milta hai.

Doctor: Yehi problem hai. Controller inhaler disease ko control karta hai aur rescue inhaler sirf temporary relief deta hai.

Patient: Accha.

Allergy History

Doctor: Naak band rehti hai?

Patient: Haan.

Doctor: Chheenk aati hai?

Patient: Bahut.

Doctor: Subah uthte hi?

Patient: Haan.

Doctor: Ghar mein dust exposure hai?

Patient: Construction ka kaam chal raha hai saamne.

Doctor: Pets hain ghar mein?

Patient: Ek cat hai.

Doctor: Symptoms cat ke aas paas badhte hain?

Patient: Shayad haan.

Previous Medical History

Doctor: Last hospitalization kab hua tha asthma ke liye?

Patient: Do saal pehle.

Doctor: ICU mein admit hona pada tha?

Patient: Nahi.

Doctor: Nebulization hui thi?

Patient: Haan emergency mein.

Doctor: Steroid tablets kab li thi last?

Patient: Last winter mein.

Social History

Doctor: Smoking karti ho?

Patient: Nahi.

Doctor: Ghar mein koi smoke karta hai?

Patient: Mere husband karte hain.

Doctor: Ghar ke andar?

Patient: Kabhi kabhi balcony mein.

Doctor: Passive smoking bhi asthma worsen kar sakti hai.

Physical Examination

Doctor: Chaliye examination karte hain.

Vital Signs

BP: 124/80 mmHg

Pulse: 94/min

Respiratory Rate: 24/min

Temperature: Normal

SpO₂: 95% on room air

Clinical Findings (spoken aloud)

Doctor: Mild respiratory distress lag raha hai.

Doctor: Patient full sentences bol pa rahi hain.

Doctor: Bilateral expiratory wheeze present.

Doctor: No crackles.

Doctor: No cyanosis.

Doctor: Peak flow pichli visit se lower lag raha hai.

Clinical Discussion

Patient: Doctor saab serious toh nahi hai na?

Doctor: Filhal emergency jaisi situation nahi lag rahi.

Patient: Accha.

Doctor: Lekin asthma definitely poorly controlled lag raha hai.

Patient: Kyun?

Doctor: Dekho:

- Rescue inhaler roz use ho raha hai.
- Night symptoms aa rahe hain.
- Exercise tolerance kam ho gaya hai.
- Wheezing present hai.

Ye sab uncontrolled asthma ki taraf point karte hain.

Differential Diagnosis

Doctor: Mere dimaag mein abhi ye possibilities hain:

1. Poorly Controlled Bronchial Asthma
2. Allergic Asthma Exacerbation
3. Allergic Rhinitis with Post-Nasal Drip
4. Viral Triggered Asthma Flare

5. Dust Exposure Related Airway Hyperreactivity

Investigations Ordered

Doctor: Main kuch tests likh raha hoon:

- Spirometry
- Peak Expiratory Flow Rate
- CBC
- Absolute Eosinophil Count
- Serum IgE
- Chest X-ray PA View
- Allergy Profile

Patient: X-ray bhi karna padega?

Doctor: Bas precaution ke liye.

Prescription

Budesonide + Formoterol Inhaler

- 2 puffs twice daily

Salbutamol Inhaler

- 2 puffs SOS for breathlessness

Montelukast 10 mg

- Once daily at bedtime

Levocetirizine 5 mg

- Once daily at night

Fluticasone Nasal Spray

- 2 sprays in each nostril once daily

Steam Inhalation

- Twice daily
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Inhaler Technique Review

Doctor: Inhaler use karke dikhao.

(Patient demonstrates)

Doctor: Dekho yahan galti ho rahi hai.

Patient: Kahan?

Doctor: Puff press karte hi immediately inhale karna hai aur 10 second breath hold karna hai.

Patient: Accha.

Doctor: Aur inhaled steroid ke baad mouth rinse karna hai.

Patient: Ye mujhe pata nahi tha.

Advice / Instructions

Doctor:

- Controller inhaler miss nahi karna.
- Dust exposure avoid karo.
- Bedroom regularly clean rakho.
- Husband ko ghar ke andar smoke nahi karna chahiye.
- Rescue inhaler ki need agar roz padne lage toh immediately contact karo.
- Peak flow diary maintain karo.
- Adequate hydration rakho.

Patient: Theek hai doctor.

Doctor: Agar severe breathlessness ho, sentence complete na kar pao, ya inhaler se relief na mile toh direct emergency jaana.

Patient: Ji.

Follow-Up Plan

Doctor: Reports lekar 2 weeks mein milna.

Patient: Zaroor.

Doctor: Tab tak medicines properly follow karna.

Patient: Thank you doctor saab.

Doctor: Allah hafiz, take care.

CASE 2 – FAMILY DOCTOR FOLLOW-UP

Osteoarthritis Knee + Obesity + Vitamin D Deficiency

(Long-form Hinglish consultation designed for medical transcription testing)

Patient Details

Patient Name: Meena Joshi

Age: 58 years

Gender: Female

Visit Type: Follow-up Visit (Known Patient)

Known History:

- Bilateral Knee Osteoarthritis for 6 years
 - Obesity
 - Vitamin D Deficiency
 - Regular patient of the clinic for over 10 years
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Doctor–Patient Conversation

Doctor: Namaste Meena ji, aaiye. Kaise chal raha hai? Pichli baar jab aayi thi tab knees mein kaafi pain tha.

Patient: Namaste doctor saab. Pain toh thoda better hua tha medicines se, lekin ab phir se badh gaya hai.

Doctor: Kab se zyada ho raha hai?

Patient: Lagbhag ek mahine se.

Doctor: Dono ghutnon mein ya ek side zyada?

Patient: Dono mein hai, lekin right knee zyada dard karta hai.

Doctor: Chalne mein dikkat?

Patient: Haan. Subah uthkar pehle 10-15 minute toh properly chal bhi nahi paati.

Doctor: Morning stiffness hoti hai?

Patient: Haan.

Doctor: Kitni der rehti hai?

Patient: 10 minute ke aas paas.

Doctor: Aur din mein?

Patient: Chalne se badh jata hai.

Symptom Exploration

Doctor: Seedhiyan chadhne mein?

Patient: Bahut problem hoti hai.

Doctor: Neeche utarne mein?

Patient: Utarne mein aur zyada.

Doctor: Squatting ya floor pe baithna?

Patient: Ab toh almost band kar diya hai.

Doctor: Koi swelling notice ki?

Patient: Kabhi kabhi shaam ko right knee thoda sujha hua lagta hai.

Doctor: Redness ya garam lagta hai?

Patient: Nahi.

Previous Treatment Discussion

Doctor: Last time jo physiotherapy suggest ki thi, continue ki?

Patient: Ek mahina ki thi.

Doctor: Fir kyun band kar di?

Patient: Thoda office ka workload badh gaya.

Doctor: Aur exercises?

Patient: Honestly doctor saab, regular nahi kar payi.

Doctor: Weight ka kya hua?

Patient: Shayad badh gaya.

Doctor: Last time kitna tha yaad hai?

Patient: 84 ya 85 ke aas paas.

Lifestyle Discussion

Doctor: Daily kitna walk karti ho?

Patient: Bas ghar ke kaam aur office.

Doctor: Dedicated walk?

Patient: Nahi.

Doctor: Office mein sitting job hai?

Patient: Haan, accounts department mein hoon.

Doctor: Pura din chair pe?

Patient: Lagbhag.

Functional Impact

Doctor: Pain ki wajah se daily life kitni affect ho rahi hai?

Patient: Bahut.

Doctor: Example?

Patient: Market jaana mushkil lagta hai. Mandir ki seedhiyan chadhna problem ho gaya hai.

Doctor: Long distance walking?

Patient: 10-15 minute se zyada nahi ho pata.

Doctor: Raat mein pain ki wajah se neend toot jati hai?

Patient: Kabhi kabhi.

Medication Review

Doctor: Calcium aur Vitamin D le rahi ho?

Patient: Vitamin D toh khatam ho gayi thi.

Doctor: Kab?

Patient: 2 mahine pehle.

Doctor: Aur renew nahi karwayi?

Patient: Nahi.

Doctor: Painkiller kitni baar lena padta hai?

Patient: Week mein 3-4 baar.

Doctor: Kaunsi?

Patient: Jo aapne di thi... Aceclofenac wali.

Associated Symptoms

Doctor: Fever?

Patient: Nahi.

Doctor: Weight loss?

Patient: Nahi.

Doctor: Chhote joints mein pain? Haath ya ungliyon mein?

Patient: Nahi.

Doctor: Morning stiffness ek ghante se zyada rehti hai?

Patient: Bilkul nahi.

Doctor: Accha.

Physical Examination

Doctor: Chaliye examination karte hain.

Vital Signs

BP: 132/84 mmHg

Pulse: 78/min

Weight: 89 kg

BMI: 33 kg/m²

Clinical Findings (spoken aloud)

Doctor: Weight 4 kilo badh gaya hai pichli visit se.

Doctor: Right knee mein mild swelling hai.

Doctor: Crepitus present hai.

(Knee movement sound)

Doctor: Flexion mildly restricted.

Doctor: No redness.

Doctor: No significant joint effusion.

Doctor: Left knee mein bhi crepitus hai but less severe.

Clinical Discussion

Patient: Doctor saab operation toh nahi karwana padega?

Doctor: Abhi immediately toh nahi lag raha.

Patient: Accha.

Doctor: Lekin weight control aur exercises nahi karoge toh future mein surgery consider karni pad sakti hai.

Patient: Samajh gayi.

Doctor: X-ray reports laaye ho?

Patient: Haan.

(Paper rustling)

Doctor: Haan. Last X-ray mein moderate osteoarthritis changes dikh rahe the.

Differential Diagnosis

Doctor: Mere hisaab se possibilities ye hain:

1. Bilateral Knee Osteoarthritis (Progressive)
 2. Mechanical Knee Pain due to Obesity
 3. Vitamin D Deficiency Related Musculoskeletal Pain
 4. Early Degenerative Joint Disease Progression
 5. Less likely Inflammatory Arthritis
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Investigations Ordered

Doctor: Kuch tests repeat karte hain:

- Vitamin D3 Level
- Serum Calcium
- CBC
- ESR

- CRP
- Knee X-ray AP & Lateral View (Both Knees)
- Blood Sugar (Fasting)

Patient: Blood sugar kyun?

Doctor: Age aur weight dono risk factors hain.

Prescription

Tab Aceclofenac 100 mg + Paracetamol 325 mg

- Twice daily after food for 5 days

Tab Pantoprazole 40 mg

- Once daily before breakfast

Tab Calcium Citrate + Vitamin D3

- Once daily after dinner

Cap Vitamin D3 60,000 IU

- Once weekly for 8 weeks

Diclofenac Gel

- Local application three times daily
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Physiotherapy Discussion

Doctor: Is baar physiotherapy seriously karni padegi.

Patient: Ghar pe kar sakti hoon?

Doctor: Haan.

Doctor: Quadriceps strengthening exercises.

Doctor: Straight leg raises.

Doctor: Hamstring stretching.

Doctor: Main sheet de deta hoon.

Lifestyle Counseling

Doctor: Sabse important cheez kya hai pata hai?

Patient: Weight kam karna?

Doctor: Bilkul.

Patient: Kitna?

Doctor: Pehle 8-10 kilo target rakho.

Patient: Itna mushkil lagta hai.

Doctor: Har 1 kilo weight loss se knee pe 3-4 kilo pressure kam hota hai.

Patient: Accha.

Doctor: Lift ki jagah stairs use kar sakti ho toh karo, lekin pain limit ke andar.

Patient: Ji.

Advice / Instructions

Doctor:

- Daily physiotherapy.
- Weight reduction.
- High-protein balanced diet.
- Long periods of sitting avoid karo.
- Floor sitting avoid karo.
- Indian toilet avoid karke western toilet use karo.
- Heavy weight lifting avoid karo.

Patient: Theek hai doctor.

Doctor: Agar knee suddenly lock ho jaye ya swelling bahut badh jaye toh immediately aana.

Patient: Ji.

Follow-Up Plan

Doctor: Reports aur X-ray lekar 3 weeks mein milna.

Patient: Zaroor.

Doctor: Aur is baar physiotherapy skip mat karna.

Patient: Promise doctor saab.

Doctor: Very good. Take care.

CASE 3 – FAMILY DOCTOR FOLLOW-UP

Vitamin B12 Deficiency + Cervical Spondylosis + Neck Pain

(Short Follow-Up Case with Multiple Medicines)

Patient Details

Patient Name: Prakash Kulkarni

Age: 52 years

Gender: Male

Occupation: Bank Employee

Known History:

- Cervical Spondylosis
 - Vitamin B12 Deficiency
 - Follow-up after treatment started 6 weeks ago
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Doctor–Patient Conversation

Doctor: Namaste Prakash ji, neck pain kaisa hai ab?

Patient: Namaste doctor saab. Pehle se better hai, lekin poori tarah gaya nahi hai.

Doctor: Pain kis side zyada hai?

Patient: Right side neck se shoulder tak jata hai.

Doctor: Haath mein jhanjhanahat abhi bhi hoti hai?

Patient: Kabhi kabhi ungliyon mein hoti hai.

Doctor: Computer pe kitne ghante kaam karte ho?

Patient: Lagbhag 8-9 ghante roz.

Doctor: Physiotherapy continue ki?

Patient: Haan doctor, week mein 3 baar gaya tha.

Doctor: Achha. Vitamin B12 wali medicine li regularly?

Patient: Ji, roz li.

Doctor: Weakness ya thakan kaisi hai?

Patient: Pehle se kaafi better hai.

Doctor: Koi naya symptom?

Patient: Nahi.

Clinical Examination

BP: 126/82 mmHg

Pulse: 78/min

Mild tenderness over cervical region

Neck movements mildly restricted on right side

Upper limb power normal

No neurological deficit

Differential Diagnosis

1. Improving Cervical Spondylosis
 2. Residual Cervical Muscle Spasm
 3. Vitamin B12 Deficiency (improving)
 4. Cervical Radiculopathy (mild)
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Investigations

No immediate investigations required.

Repeat Vitamin B12 level after 2 months.

Prescription

Tab Methylcobalamin 1500 mcg

- Once daily after dinner for 2 months

Tab Etoricoxib 60 mg

- Once daily after food for 5 days (if pain increases)

Muscle Relaxant (Thiocolchicoside 4 mg)

- Twice daily for 5 days

Diclofenac Gel

- Local application three times daily
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Advice

Doctor: Laptop screen eye level pe rakho.

Patient: Ji.

Doctor: Har 45 minute mein neck stretching karni hai.

Patient: Theek hai.

Doctor: Mobile use karte waqt neck neeche jhukakar zyada der mat baithna.

Patient: Ye aadat hai meri.

Doctor: Isi wajah se aadhe patients mere paas aate hain. *(laughs)*

Patient: *(laughs)* Samajh gaya doctor.

Doctor: Physiotherapy continue rakho aur agar haath mein weakness ya severe numbness ho toh immediately dikhana.

Patient: Zaroor.

Doctor: 2 months baad follow-up karte hain.

Patient: Thank you doctor saab.

CASE 1 – NEW PATIENT INTERACTION

Persistent Cough, Weight Loss & Suspected Pulmonary Tuberculosis

(Long-form Hinglish consultation designed for healthcare transcription testing)

Patient Registration

Doctor: Namaste, aaiye baithiye.

Patient: Namaste doctor.

Doctor: Pehli baar aaye hain?

Patient: Ji.

Doctor: Theek hai, pehle details note kar lete hain.

Doctor: Aapka naam?

Patient: Imran Shaikh.

Doctor: Umar?

Patient: 34 saal.

Doctor: Kya kaam karte hain?

Patient: Main electrician hoon.

Doctor: Married?

Patient: Ji.

Doctor: Kahan rehte hain?

Patient: Hadapsar, Pune.

Doctor: Kisi emergency contact ka number?

Patient: Mere bhai ka de deta hoon.

Chief Complaint

Doctor: Toh Imran ji, kya problem hai?

Patient: Doctor saab, lagbhag ek mahine se khansi ja hi nahi rahi.

Doctor: Ek mahina?

Patient: Ji.

Doctor: Dry khansi hai ya balgam ke saath?

Patient: Balgam ke saath.

History of Present Illness

Doctor: Balgam ka colour kaisa hota hai?

Patient: Safed ya halka peela.

Doctor: Kabhi khoon aaya?

Patient: Do baar thoda sa laal rang dikha tha.

Doctor: Theek. Fever hota hai?

Patient: Haan, especially shaam ko.

Doctor: Roz?

Patient: Lagbhag roz.

Doctor: Temperature check kiya?

Patient: Nahi.

Doctor: Raat ko pasina aata hai?

Patient: Bahut zyada.

Doctor: Kapde badalne padte hain?

Patient: Kabhi kabhi haan.

Doctor: Weight mein koi change?

Patient: Pichhle 2 mahine mein lagbhag 6 kilo kam ho gaya.

Doctor: Diet kar rahe the?

Patient: Nahi.

Doctor: Bhook kaisi hai?

Patient: Kam ho gayi hai.

Doctor: Saans phoolti hai?

Patient: Seedhiyan chadhne pe.

Doctor: Chest pain?

Patient: Kabhi kabhi khansi ke saath.

Previous Treatment History

Doctor: Iske liye pehle kahin dikhaya tha?

Patient: Haan, local clinic mein.

Doctor: Kya medicines mili thi?

Patient: Antibiotic aur cough syrup diya tha.

Doctor: Kitne din li?

Patient: 5 din.

Doctor: Improvement hua?

Patient: Bilkul nahi.

Past Medical History

Doctor: Pehle kabhi TB hui thi?

Patient: Nahi.

Doctor: Diabetes?

Patient: Nahi pata.

Doctor: Asthma?

Patient: Nahi.

Doctor: HIV ya koi immune problem?

Patient: Nahi.

Doctor: Hospital admission kabhi hua?

Patient: Nahi.

Contact History

Doctor: Ghar ya kaam par kisi ko TB hui hai?

Patient: Mere chacha ko 2 saal pehle hui thi.

Doctor: Aap unke contact mein the?

Patient: Haan, kuch mahine saath rehte the.

Doctor: Treatment complete hua tha unka?

Patient: Haan.

Social History

Doctor: Smoking karte ho?

Patient: Haan.

Doctor: Kitni?

Patient: 8-10 cigarette roz.

Doctor: Kitne saal se?

Patient: 15 saal.

Doctor: Alcohol?

Patient: Kabhi kabhi.

Occupational History

Doctor: Kaam mein dust exposure hota hai?

Patient: Construction sites pe kaam karta hoon toh dust rehti hai.

Doctor: Mask use karte ho?

Patient: Hamesha nahi.

Review of Systems

Doctor: Koi neck swelling?

Patient: Nahi.

Doctor: Joint pain?

Patient: Nahi.

Doctor: Persistent fatigue?

Patient: Haan.

Doctor: Daily kaam karne mein weakness lagti hai?

Patient: Ji.

Physical Examination

Vital Signs

BP: 118/72 mmHg

Pulse: 102/min

Temperature: 100.2°F

Respiratory Rate: 22/min

SpO₂: 96%

Weight: 59 kg

Clinical Findings (spoken aloud)

Doctor: Patient visibly thin lag rahe hain.

Mild pallor present.

Low-grade fever present.

Chest examination mein right upper lung zone mein crackles sunai de rahe hain.

No pedal edema.

Clinical Discussion

Patient: Doctor saab, ye normal infection hai kya?

Doctor: Honestly, mujhe kuch serious causes bhi consider karne padenge.

Patient: Jaise?

Doctor: TB ko rule out karna bahut important hai.

Patient: TB?

Doctor: Haan, kyunki aapko:

- Ek mahine se khansi hai
- Weight loss hai
- Evening fever hai
- Night sweats hain
- Aur blood-streaked sputum bhi hua hai

Patient: Samajh gaya.

Differential Diagnosis

1. Pulmonary Tuberculosis
 2. Chronic Lung Infection
 3. Community Acquired Pneumonia (Partially Treated)
 4. Bronchiectasis
 5. Lung Malignancy (Less Likely but Must Be Ruled Out)
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Investigations Ordered

Urgent Tests

- Chest X-Ray PA View
 - Sputum for AFB (2 Samples)
 - CBNAAT / GeneXpert
 - Complete Blood Count
 - ESR
 - CRP
 - HIV Screening (after counseling and consent)
 - Blood Sugar
-

Prescription (Symptomatic Until Diagnosis Confirmed)

Tab Paracetamol 650 mg

- SOS for fever

Cough Syrup (Ambroxol + Guaifenesin)

- 10 ml three times daily

Multivitamin Capsule

- Once daily
-

Counseling

Doctor: Main abhi TB ki medicines start nahi karunga.

Patient: Kyun?

Doctor: Pehle diagnosis confirm karna zaroori hai.

Patient: Theek hai.

Doctor: Tests jaldi kara lo.

Patient: Kal hi karwa lunga.

Advice / Instructions

Doctor: Jab tak reports nahi aati:

- Khanshte waqt mouth cover karo.
- Separate towel use karo.
- Smoking immediately band karo.
- Adequate nutrition lo.
- Paani zyada piyo.

Patient: Ji doctor.

Doctor: Agar breathing suddenly worsen ho ya zyada blood aaye sputum mein toh immediately hospital jaana.

Patient: Theek hai.

Follow-Up Plan

Doctor: Reports lekar 2-3 din mein milna.

Patient: Zaroor.

Doctor: Diagnosis confirm hote hi treatment start karenge.

Patient: Thank you doctor saab.

Doctor: Take care.

CASE 2 – NEW PATIENT INTERACTION

Abdominal Pain + Gallstones (Cholelithiasis) vs Acute Cholecystitis

(Long-form Hinglish consultation designed for healthcare transcription testing)

Patient Registration

Doctor: Namaste madam, aaiye baithiye.

Patient: Namaste doctor.

Doctor: Aap pehli baar hamare clinic mein aayi hain?

Patient: Ji doctor.

Doctor: Theek hai, pehle details note kar lete hain.

Doctor: Aapka poora naam?

Patient: Sunita Jadhav.

Doctor: Umar?

Patient: 43 saal.

Doctor: Occupation?

Patient: Main school teacher hoon.

Doctor: Married?

Patient: Ji.

Doctor: Kitne bachche hain?

Patient: Do.

Doctor: Address?

Patient: Pimpri, Pune.

Chief Complaint

Doctor: Toh Sunita ji, aaj kis problem ke liye aayi hain?

Patient: Doctor saab, pet mein bahut tez dard ho raha hai pichhle 3 din se.

Doctor: Kahan dard ho raha hai?

Patient: Right side, yahan upar. (*Right upper abdomen ki taraf ishara karti hain*)

History of Present Illness

Doctor: Dard achanak shuru hua ya dheere dheere?

Patient: Pehle halka tha, phir achanak bahut badh gaya.

Doctor: Constant rehta hai ya aata-jaata hai?

Patient: Waves mein aata hai, lekin poori tarah jaata nahi.

Doctor: Dard khane ke baad badhta hai?

Patient: Haan, especially oily food ke baad.

Doctor: Koi specific incident ya meal yaad hai?

Patient: Do din pehle shaadi mein gayi thi. Wahan bahut oily khana khaya tha.

Doctor: Uske baad dard shuru hua?

Patient: Haan.

Doctor: Dard shoulder ya back mein jata hai?

Patient: Haan, right shoulder tak jata hai.

Doctor: Nausea?

Patient: Bahut.

Doctor: Vomiting?

Patient: Do baar hui.

Doctor: Fever?

Patient: Kal raat se halka bukhari hai.

Pain Assessment

Doctor: Agar 0 se 10 ke scale par rate karna ho toh kitna dard hai?

Patient: 8 ya 9.

Doctor: Chalne phirne se badhta hai?

Patient: Haan.

Doctor: Rest se relief milta hai?

Patient: Thoda.

Past Medical History

Doctor: Pehle bhi aisa dard hua hai?

Patient: Haan doctor.

Doctor: Kab?

Patient: Pichhle 1 saal mein 3-4 baar hua.

Doctor: Tab kya kiya tha?

Patient: Painkiller le li thi.

Doctor: Ultrasound kabhi hua tha?

Patient: Nahi.

Doctor: Diabetes?

Patient: Nahi.

Doctor: BP?

Patient: Normal rehta hai.

Medication History

Doctor: Abhi koi regular medicines?

Patient: Nahi.

Doctor: Koi allergy medicines se?

Patient: Nahi.

Surgical History

Doctor: Koi operation hua hai pehle?

Patient: Nahi.

Family History

Doctor: Family mein kisi ko gallstones hue hain?

Patient: Haan, meri mummy ka gallbladder operation hua tha.

Doctor: Accha.

Dietary History

Doctor: Oily food kitna khati hain?

Patient: Weekend pe bahar ka kha lete hain.

Doctor: Weight gain hua hai recent years mein?

Patient: Haan.

Doctor: Kitna weight hai?

Patient: Around 78 kilo.

Review of Systems

Doctor: Jaundice hua kabhi?

Patient: Nahi.

Doctor: Urine dark hua hai?

Patient: Nahi.

Doctor: Stool ka colour normal hai?

Patient: Haan.

Doctor: Weight loss?

Patient: Nahi.

Physical Examination

Vital Signs

BP: 124/80 mmHg

Pulse: 96/min

Temperature: 100.1°F

Respiratory Rate: 18/min

Weight: 78 kg

Clinical Findings (spoken aloud)

Doctor: Mild fever present.

Abdominal examination karte hain.

Patient: Ji.

(Examination performed)

Doctor: Right upper abdomen mein tenderness hai.

Murphy's sign positive lag raha hai.

No abdominal guarding.

No jaundice visible.

Clinical Discussion

Patient: Doctor saab kya lag raha hai?

Doctor: Mujhe gallbladder ki problem lag rahi hai.

Patient: Stone?

Doctor: Ho sakta hai.

Patient: Serious hai kya?

Doctor: Abhi tests ke bina confirm nahi kar sakte, lekin gallstones ya gallbladder infection dono possibilities hain.

Differential Diagnosis

1. Symptomatic Cholelithiasis (Gallstones)
 2. Acute Cholecystitis
 3. Biliary Colic
 4. Gastritis
 5. Peptic Ulcer Disease (Less Likely)
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Investigations Ordered

Urgent Tests

- Ultrasound Abdomen
 - CBC
 - Liver Function Test
 - Serum Bilirubin
 - Serum Amylase
 - Serum Lipase
 - Blood Sugar
-

Prescription

Tab Pantoprazole 40 mg

- Once daily before breakfast

Tab Drotaverine 80 mg

- Three times daily for abdominal pain

Tab Paracetamol 650 mg

- SOS for fever

Ondansetron 4 mg

- SOS for nausea/vomiting
-

Counseling

Doctor: Sabse important test ultrasound hai.

Patient: Aaj hi karwa leti hoon.

Doctor: Bahut achha.

Doctor: Jab tak report nahi aati, oily aur fried food bilkul avoid karna.

Patient: Ji.

Doctor: Light diet rakho.

Advice / Instructions

Doctor:

- Oily food avoid.
- Adequate hydration.
- Ultrasound within 24 hours.
- Agar severe pain, high fever ya repeated vomiting ho toh emergency jaana.

Patient: Theek hai doctor.

Doctor: Agar gallstones confirm hue aur symptoms recurrent rahe toh surgery consult karna pad sakta hai.

Patient: Samajh gayi.

Follow-Up Plan

Doctor: Ultrasound report ke saath kal ya parson milna.

Patient: Zaroor.

Doctor: Tab final treatment plan decide karenge.

Patient: Thank you doctor.

Doctor: Take care.

CASE 3 – NEW PATIENT

Acute Gastritis

Registration

Doctor: Naam?

Patient: Kunal Deshmukh.

Doctor: Age?

Patient: 31 years.

Doctor: Occupation?

Patient: Sales executive.

Chief Complaint

Patient: Pet mein jalan aur acidity ho rahi hai.

History

Doctor: Kab se?

Patient: 5 din se.

Doctor: Khane ke baad badhta hai?

Patient: Haan.

Doctor: Vomiting?

Patient: Ek baar hui.

Doctor: Alcohol ya spicy food?

Patient: Weekend pe dono zyada ho gaya tha.

Examination

BP: 124/80 mmHg

Pulse: 82/min

Mild epigastric tenderness

Differential Diagnosis

1. Acute Gastritis
 2. Acid Peptic Disease
 3. GERD
-

Prescription

Tab Pantoprazole 40 mg

- Before breakfast

Antacid Syrup

- 10 ml TDS

Tab Ondansetron 4 mg

- SOS
-

Advice

Spicy food avoid karo.

Alcohol avoid karo.